CONTENT BANK · READY TO SCRIPT

10 Pillars · 50 Subtopics · 150 Angles

Content for Dr. Azka's own account, written to attract clinic-owner doctors (dermatologists, cosmetic dentists, aesthetic physicians), not patients. Every hook speaks to the doctor, every talking point teaches them something useful, and every CTA brings them to you. Every angle includes the spoken hook, on-screen text, 3 talking points, CTA, format and funnel stage, so each one can go straight into a script.

Sources: your positioning, ICP, business model, funnel/offer and tonality documents (Files 1–7) · 210 keyword-research reels across 21 keyword clusters · 76 competitor reels (Amra Farooq, Sam R Patel, Dr. Nickc, Maha Chalawi, Mrinal Tiwari, Akash Gupta, Shivam Bansal, Joshua Navaro, Dr Sakshi, Jakeyla Reed, Eshaan).
Audience (who you're talking to): clinic-owner dermatologists, cosmetic dentists and aesthetic physicians · UAE + USA · Prepared 31 October 2026

How to read each angle

1.1.1 The spoken hook: the first words out of your mouth (0–1.5 s)

On-screen text: the big written hook in frame 1

- Talking point 1: the stakes or the first idea
- Talking point 2: the payload (steps / demo / example)
- Talking point 3: the proof or payoff line

CTA: the one action you ask for

Format · STAGE

Funnel stages: TOFU discovery: doctors find you and follow · MOFU trust: clinic owners see you as the expert · BOFU conversion: doctors DM you, book audits and calls.

'Built on' under each subtopic shows which keyword cluster or competitor reels it comes from, and the gap it fills.

Pillars and content mix

#	Pillar	Share of posts
1	What Your Clinic Should Post (Content Strategy for Doctors)	15%
2	Hooks, Scripting & Filming for Busy Doctors	10%
3	Views → Patients: Turning Instagram Into a Patient Channel	15%
4	Doctor Personal Branding & Authority (Without Becoming an Influencer)	10%
5	Clinic Branding: Build Your Clinic Around Its Specialists	10%
6	Compliant & Evidence-Based Marketing for Doctors (UAE + USA)	5%
7	The Patient Leak: Why Clinics Lose Inquiries Before They Book	10%
8	AI Systems for Clinics: Booking, No-Shows & 24/7 Response	15%
9	Retention, Reactivation & Clinic Growth Numbers	5%
10	Dr. Azka: A Doctor Building for Doctors (Story, Proof & Behind the Scenes)	5%

Scripting rule for every angle (value per second)

Hook in 1.5 s, with visual + text + voice saying the same thing → stakes → payload, one idea per line → proof → one CTA. Every sentence must be a claim, a number, an example or a step. If you can delete a sentence and nothing is lost, delete it.

Compliance

Angles that mention regulators or rules (DHA, DoH, MOHAP, UAE advertiser permits, FTC, HIPAA, state boards, US consent rules) are educational. Check each one against the regulator's current guidance before filming, and add 'not legal advice' to the caption. Use real patient images or stories only with written consent.

PILLAR 1 OF 10 · 15% OF POSTS

What Your Clinic Should Post (Content Strategy for Doctors)

Your biggest authority pillar. Doctors learn exactly what to post for their specialty, and see that you understand their world.

Research basis: Keyword clusters: 'medical content marketing', 'doctor content strategy', 'Dermatologist content ideas', 'Dentists Instagram marketing', 'Medspa/aesthetic social media marketing'. Competitors: Amra Farooq, Akash Gupta ('5 pillars'), 'Content ideas for a med spa / dentists'. Their gap: generic 'doctors', no finished specialty examples, ideas locked behind 'comment CONTENT', and mostly India-focused.

1.1 Specialty content blueprints

Built on: 'If I had to take a Dentist... 30 days' and 'Content ideas for dentists / med spa'. Their gap: dense lists, no sample script or calendar. You give the finished blueprint.

1.1.1 Dermatologists: if I ran your Instagram this month, these are the only 5 videos I'd make you film.

On-screen: “5 videos for every dermatologist”

- 1) Your honest 'what I'd do for my own skin' video. It builds trust faster than any treatment post.
- 2) A consultation walkthrough. 3) 'Who is NOT a candidate'. 4) One myth you correct every week in clinic.
- 5) A recovery-day reality video for your highest-value treatment. Show the hook for each on screen.

CTA: Comment DERM and I'll send you the 5 scripts.

Talking head + list · TOFU

1.1.2 Cosmetic dentists: 4 videos that sell veneers without you ever posting a price.

On-screen: “Sell veneers. Never post a price.”

- Video 1: close-up 'natural vs. fake' veneers. It removes your patients' biggest fear.
- Video 2: your consultation in 40 seconds. Video 3: who you'd turn away.
- Video 4: a patient-journey day-by-day (with consent). Price is handled in the DM, where you can qualify.

CTA: Comment VENEERS for the 4 scripts.

Green-screen + examples · MOFU

1.1.3 Aesthetic doctors: one simple series format that can fill your consultation calendar.

On-screen: “1 series format. Fuller calendar.”

- The '30-second treatment explained' series: what it is → who it's for → feel → downtime → one honest limitation.
- Show it applied to Botox, Morpheus8 and skin boosters.
- Why series work: patients binge them, so your profile visits and DMs go up.

CTA: Comment SERIES and I'll send the template.

Template breakdown · MOFU

1.2 Turning your expertise into content

Built on: Amra Farooq 'patients connect with expertise, not ChatGPT scripts' and 'Content ideas for a doctor's office (answer embarrassing questions)'. Your gap: a repeatable system that pulls ideas from the doctor's own clinic.

1.2.1 Doctors: you already have 100 content ideas. You just hear them in clinic every day.

On-screen: “Your clinic = your content library”

- Every question a patient asks you twice is a video.
- Have your front desk log FAQs for 2 weeks; that's a month of content.
- Show a sample FAQ list turned into 10 hooks.

CTA: Comment IDEAS for the FAQ-to-content sheet.

Talking head · TOFU

1.2.2 Stop explaining treatments like a textbook. Explain them like you do in your consultation room.

On-screen: “Textbook ❌ Consultation ✅”

- Patients book the doctor who makes them feel understood, not the one with the longest explanation.
- Side by side: a textbook script vs. a 'consultation voice' script.
- The rule: patient's worry first, medicine second.

CTA: Save this before your next filming day.

Split screen · TOFU

1.2.3 How to turn one treatment you offer into a month of content.

On-screen: “1 treatment = 30 days”

- Break it into: myths, candidacy, pain, downtime, results timeline, cost factors, aftercare, mistakes, your philosophy.
- Each one is 2–3 videos.
- Show the full content tree on screen.

CTA: Comment TREE and I'll build one for your main treatment.

Diagram · MOFU

1.3 What NOT to post

Built on: 'Stop putting your patients to sleep with the same 5 Symptoms videos', 'Bad / Good / Excellent'. Their gap: no WHY and no replacement. You give the swap.

1.3.1 5 posts that make your clinic look like every other clinic, and what to post instead.

On-screen: “5 posts to stop today”

- 'Happy Monday' graphics, stock photos, discount flyers, product hauls, logo-only posts.
- Why they fail: any clinic could post them, so they prove nothing about you.
- One replacement video for each.

CTA: Comment SWAP for all 5 replacements.

List + examples · TOFU

1.3.2 Doctors: your agency's flyers are hurting your reputation. Here's why.

On-screen: “Flyers ≠ authority”

- Flyers say 'we have a designer'. Videos of you say 'we have expertise'.
- Patients choose doctors, not designs.
- Show 3 anonymised flyers and the video that should replace each.

CTA: Send this to whoever runs your Instagram.

Before/after posts · TOFU

1.3.3 Your 'meet the team' post gets likes but no DMs. Here's the 20-second fix.

On-screen: “Likes ≠ DMs”

- Team photos create warmth but give patients no reason to book.
- Fix: each team member answers one patient question on camera.
- Now the same post builds trust and handles objections.

CTA: Try it this week and tag me.

Teardown · MOFU

1.4 Profile teardowns

Built on: your File 5 tonality reel ('This is how I grow this dermatologist...'), a teardown format that works. Use permission or anonymise.

1.4.1 I audited a Dubai dermatologist's Instagram. Here are the 3 things costing them patients.

On-screen: “Audit: 3 lost-patient mistakes”

- Bio fails the 5-second rule. No doctor on camera. No clear next step.
- Show the fix for each, live.
- Anonymise or get permission.

CTA: Comment AUDIT and I'll review your profile. Green-screen teardown · MOFU

1.4.2 Rating 3 cosmetic dentists' Instagram profiles out of 10.

On-screen: “3 dentist profiles. Rated.”

- The same 5 criteria for each: bio, doctor visibility, content mix, proof, CTA.
- Show why the top scorer wins.
- The single biggest fix for the others.

CTA: Comment RATE for yours. Ranking · TOFU

1.4.3 This US aesthetic clinic has 40k followers and almost no booking path. Let me show you.

On-screen: “40k followers. No path to booking.”

- Followers without a path to booking = wasted attention.
- Map what a patient sees from Reel → profile → DM → booking.
- Three fixes any clinic can copy.

CTA: Follow for weekly clinic teardowns. Screen recording · MOFU

1.5 Seasonal planning for clinic owners (UAE vs. USA)

Built on: 'Clinic content calendar' cluster ('Plan a month of content in 45 minutes'). Their gap: no market-specific seasonality. You plan for Dubai AND the US.

1.5.1 UAE clinic owners: the 3 content seasons most clinics miss every year.

On-screen: “UAE: 3 seasons clinics miss”

- Wedding season prep (start 3–6 months out), Ramadan (respectful, routine-change content), pre-Eid.
- Summer heat: pigmentation and skin concerns spike, so plan 4–6 weeks before.
- Why posting in the season is too late.

CTA: Comment UAE for the 12-month calendar. Carousel calendar · MOFU

1.5.2 US clinic owners: plan these 4 content windows 6 weeks early, or miss the bookings.

On-screen: “US: 4 booking windows”

- January resolutions, pre-summer, fall laser season, holiday-party treatments.
- Content needs 4–6 weeks to build intent before patients book.
- Show the 12-month grid.

CTA: Comment USA for the grid. Calendar walkthrough · MOFU

1.5.3 How I plan a full month of content for a doctor in one 45-minute meeting.

On-screen: “1 meeting = 1 month”

- Agenda: business goals → treatments to push → FAQs → seasonal moments → filming dates.
- Output: 40 topics, mapped to pillars and funnel stages.
- This is exactly what my clients get.

CTA: DM PLAN if you want me to plan yours. Screen recording · BOFU

PILLAR 2 OF 10 · 10% OF POSTS

Hooks, Scripting & Filming for Busy Doctors

Shows doctors why their videos die in 3 seconds and how to film without losing hours. It proves the exact skill they'd pay you for.

Research basis: Keyword clusters: 'doctor content strategy', 'Doctors personal branding' ('Reel hooks for doctors'). Competitors: Akash Gupta (QUEST, 'comment HOOK', hooks withheld), Amra Farooq (visual hooks, ChatGPT vs. authentic), 'How to script reels as a doctor'. Their gap: no worked rewrites, frameworks without examples.

2.1 Hook engineering for doctors

Built on: Akash Gupta 'scroll-stopping hooks' (promised 20 hooks, showed none). Your gap: show the hooks and why they work.

2.1.1 Doctors: your hook isn't your first sentence. It's the first frame, word and text, all at once.

On-screen: “Hook = Visual + Written + Verbal”

- Visual: movement or a clinical prop in frame 1. Written: 5–7 specific words. Verbal: the same promise in under 2 seconds.
- Show a doctor's video where all three mismatch.
- Then the fixed version.

CTA: Save this for your next filming day.

Visual breakdown · TOFU

2.1.2 I rewrote a dermatologist's hook 5 ways. Here's the one that would actually stop patients.

On-screen: “1 hook, 5 rewrites”

- Original: 'Today let's talk about acne.'
- Rewrites: number, fear, myth, POV, result-first. Which suits reach and which suits bookings.
- The winner and why: specificity + the patient's own words.

CTA: Comment HOOK and I'll rewrite yours.

Split screen · TOFU

2.1.3 Doctors, stop starting your videos with 'Hi, I'm Dr...'. Say this instead.

On-screen: “Delete your intro”

- Your name is in the caption and profile. The first second belongs to the patient's problem.
- Formula: patient's thought + surprising answer.
- 3 examples: derm, dental, aesthetics.

CTA: Follow for one hook fix a day.

Talking head · TOFU

2.2 Script surgery

Built on: 'How to script reels as a doctor' (long melanoma example) and 'ChatGPT-sounding scripts'. Your gap: cut a real doctor script on screen.

2.2.1 This doctor's 60-second script had 22 seconds of filler. Watch me cut it.

On-screen: “60s → 38s”

- Highlight filler: greetings, 'basically', repeating the question, the long outro.
- Apply the delete test sentence by sentence.
- Play both versions: the tighter one sounds more expert.

CTA: Comment SCRIPT and send me yours.

Screen recording · MOFU

2.2.2 The 4-part script structure I use for every doctor client.

On-screen: “Hook → Stakes → Payload → Next step”

- Hook: the patient's thought. Stakes: why it matters now.
- Payload: 3 steps or 1 demo, one idea per line. Next step: one CTA.
- A filled-in example on screen.

CTA: Screenshot this template.

Template on screen · MOFU

2.2.3 Why your ChatGPT-written scripts sound like every other doctor's.

On-screen: “ChatGPT voice ❌ Your voice ✅”

- AI scripts are generic, can include wrong facts, and miss local rules.
- Use AI for structure; add your cases, your opinions, your words.
- Rewrite one AI script live into a doctor's real voice.

CTA: Follow for scripts that sound like you.

Split screen · TOFU

2.3 Retention: why patients stop watching your videos

Built on: Akash Gupta QUEST (no worked example), the 'Memorable medical content' thought-experiment reel. Your gap: show where viewers drop off and the fix.

2.3.1 Doctors, your videos lose most viewers at second 7. Here's why.

On-screen: “Second 7 = the drop”

- After the hook, doctors 'give context'. That's where people leave.
- Fix: open a loop ('the third one is what most patients get wrong').
- Show a retention graph with the drop and the fix.

CTA: Follow. Part 2 is pacing.

Talking head + graphic · TOFU

2.3.2 Your on-screen text is repeating what you say. Here's what it should do.

On-screen: “Text ≠ subtitles”

- Captions carry the words; big text should carry structure (1/3, ❌/✅, numbers).
- Show both versions side by side.
- This helps people watching on mute, who are a big share of viewers.

CTA: Send this to your editor.

Demo · TOFU

2.3.3 Doctors over-explain. The 'one idea per breath' rule fixes it.

On-screen: “One idea. One breath.”

- You're trained to be thorough; Reels reward clarity.
- Each sentence must be a claim, a number, an example or a step.
- Split one long doctor sentence into 3 breaths, live.

CTA: Tag a doctor who over-explains 😄

Talking head · TOFU

2.4 Filming without losing your clinic days

Built on: 'The I don't have time objection is real', Dr. Nickc 'how to make content as a dentist with limited time'. Their gap: no workflow or realistic time numbers.

2.4.1 How a busy dermatologist can film 40 videos in two 3-hour sessions a month.

On-screen: “40 videos. 6 hours.”

- Pre-written scripts, a teleprompter, 3 outfits, one background per batch.
- Order: easy talking heads first, B-roll last.
- Between sessions, the doctor does nothing. That's my team's job.

CTA: Comment BATCH for the filming-day checklist.

BTS of filming day · MOFU

2.4.2 Doctors, you don't need a studio. Here's my exact filming setup for under $300.

On-screen: “Clinic → studio for <$300”

- Phone + lav mic + one soft light + the clinic background.
- Where to stand, which light, which wall.
- Show the clinic before and after the setup.

CTA: Save this setup list.

Demo · TOFU

2.4.3 The filming-day mistakes that waste half of a doctor's footage.

On-screen: “5 filming mistakes”

- Bad audio, mixed lighting, no B-roll, reading off-camera, filming without scripts.
- A fix for each in one line.
- Good footage = cheaper, faster editing.

CTA: Send this to your clinic's content person.

List · TOFU

2.5 Camera confidence

Built on: 'Camera confidence for doctors (Confidence comes after execution)'. Their gap: no exercise. Your File 1 lists 'removing the fear of camera' as a topic you love.

2.5.1 Doctors who hate the camera: film in 'consultation mode', not 'presenter mode'.

On-screen: “Talk to ONE patient”

- You're great with one patient but freeze when 'presenting'.
- Trick: someone behind the camera asks the question; you answer them.
- Show the stiff take vs. the consultation-mode take.

CTA: Try it in your next session and tell me.

Talking head · TOFU

2.5.2 You don't need to be charismatic on camera, Doctor. You need to be clear.

On-screen: “Clear > charismatic”

- Patients want safety and clarity, not entertainment.
- 3 habits: short sentences, look at the lens, one point per video.
- Your calm doctor voice IS the brand.

CTA: Send this to a doctor who says 'I'm not a camera person'.

Talking head · TOFU

2.5.3 A 5-minute warm-up I give every doctor before filming.

On-screen: “5-minute camera warm-up”

- Voice warm-up, 3 practice takes on an easy topic, posture check.
- Why the first 3 takes are always the worst.
- Show a doctor's take 1 vs. take 4.

CTA: Save this for filming day.

Demo · TOFU

PILLAR 3 OF 10 · 15% OF POSTS

Views → Patients: Turning Instagram Into a Patient Channel

Separates you from 'grow your Instagram' creators. Doctors want booked consultations, not followers.

Research basis: Keyword clusters: 'patient acquisition strategies', 'patient inquiry conversion', 'Clinic lead generation / marketing ROI'. Competitors: Amra Farooq ('relevance over reach', 'patients may already be watching'), Joshua Navaro (Orange County case study), Eshaan (profile visitors). Their gap: no conversion data, no attribution method, no audits.

3.1 Views vs. patients

Built on: Amra Farooq 'A reel can get millions of views and still generate zero patients' (no example or data). You add the numbers and a pre-posting check.

3.1.1 Doctors, a 200k-view Reel with zero consultations is a failed Reel.

On-screen: “200k views. 0 patients.”

- Three questions before you post: Is it for my local patient? Does it show my expertise? Is there a next step?
- Viral topics bring global followers; treatment content brings bookers.
- The checklist on screen.

CTA: Save the checklist.

Talking head · TOFU

3.1.2 The 3 numbers I check for doctors before I even look at views.

On-screen: “Views come last”

- Profile visits = curiosity. DMs = intent. Link taps = action.
- Show Instagram Insights on a demo account.
- A Reel with fewer views but more DMs is the better Reel.

CTA: Follow for the full metrics series.

Insights screen recording · MOFU

3.1.3 Your viral Reel brought followers from 40 countries and no patients from your city.

On-screen: “Wrong audience”

- Broad topics = broad audience. Fine for reach, weak for bookings.
- Fix: localise ('in Dubai's humidity...'), name your treatment, add a local CTA.
- The right mix: some reach videos, most local-intent videos.

CTA: Comment LOCAL for 10 localised hooks.

Talking head · TOFU

3.2 Your profile is your sales page

Built on: your File 5 reel ('bio doesn't pass the 5-second rule'), 'Most doctors lose potential patients before they watch a Reel'. Their gap: no before/after profiles.

3.2.1 Doctors, your bio fails the 5-second test. Here's the line-by-line fix.

On-screen: “Bio: before → after”

- L1: name + specialty + city (searchable). L2: the transformation you deliver.
- L3: proof (years, cases, credentials). L4: one CTA.
- Rewrite a real-style bio live.

CTA: Comment BIO and I'll rewrite yours.

Profile teardown · MOFU

3.2.2 The 5 highlights every aesthetic or dental clinic needs.

On-screen: “5 highlights = 1 sales page”

- Results (with consent), treatments explained, meet the doctor, FAQs and price policy, reviews.
- Messy highlights vs. organised ones.
- Most clinics can fix this in an afternoon.

CTA: Save this and fix yours tonight. Screen recording · MOFU

3.2.3 Pin these 3 videos on your clinic's profile, Doctor.

On-screen: “Pin these 3”

- Pin 1: who you are and why patients trust you. Pin 2: your signature treatment explained.
- Pin 3: a result story or review compilation.
- New visitors decide in seconds.

CTA: Follow for more profile fixes. Talking head · MOFU

3.3 CTAs that make patients act

Built on: competitor CTA analysis. Most reels in your data have no clear CTA or a vague one. Your gap: specific CTAs that continue the value.

3.3.1 Doctors, 'link in bio' is costing you patients. Use this instead.

On-screen: “Link in bio ❌ Keyword ✅”

- Comment keywords trigger an instant DM, which starts a conversation.
- Conversations convert better than cold links.
- Show the comment → DM → question → booking flow.

CTA: Comment KEYWORD and watch what happens 😉 Demo · MOFU

3.3.2 When should a doctor ask for the booking in a video, and when shouldn't they?

On-screen: “Soft CTA vs. hard CTA”

- Discovery content: soft (save, follow). Treatment and proof content: hard (book, DM).
- Asking on every post feels salesy; never asking wastes intent.
- A simple rule for each post type.

CTA: Save this rule. Talking head · MOFU

3.3.3 Here's the comment → reply → booking flow I set up for clinics.

On-screen: “Comment → Reply → Booked”

- Screen-record the demo: comment 'LASER' → DM in seconds → 2 questions → 2 time slots.
- Where the AI stops and your team takes over.
- This is where content meets systems.

CTA: Comment FLOW if you want it for your clinic. Screen recording · BOFU

3.4 Tracking: which video brought which patient

Built on: Joshua Navaro (ROI without attribution), Dr. Nickc KPI teaser. Your gap: show HOW to know what works.

3.4.1 Doctors, here's how to know which Reel brought which patient, without expensive software.

On-screen: “Which Reel = which patient?”

- Unique comment keywords per Reel; 'how did you hear about us?' at intake.
- A simple sheet: date, source, Reel, treatment, booked, value.
- Review monthly and make more of what books.

CTA: Comment SHEET for the template. Talking head + sheet · MOFU

3.4.2 The one question your front desk must ask every new patient.

On-screen: “'How did you hear about us?'”

- Without it, every marketing decision you make is a guess.
- Make it a required field in booking and intake.
- Be specific: 'Instagram, which video?'

CTA: Send this to your front desk. Talking head · MOFU

3.4.3 What your agency should report every month, and it's not 'we posted 40 Reels'.

On-screen: “Activity ≠ results”

- Content metrics, patient-acquisition metrics, follow-up metrics.
- Lead with inquiries, bookings and reactivated patients.
- Show my client report (dummy data).

CTA: Comment REPORT for the template. Report walkthrough · BOFU

3.5 Why patients choose another clinic

Built on: Shivam Bansal ('the brain chooses who feels safe'), 'Two doctors, same city, same skills', Jakeyla Reed (pre-sell before the consultation). Your gap: you're a doctor explaining patient psychology to doctors.

3.5.1 Two doctors, same city, same skills. Here's why patients book the other one.

On-screen: “Same skills. Different bookings.”

- Patients can't judge clinical skill; they judge clarity, familiarity and safety.
- Visibility + trust content = 'I already know this doctor'.
- What to post so you're the familiar one.

CTA: Follow if you're the better doctor but the less visible one. Talking head · TOFU

3.5.2 Your patients watch you for weeks before they DM. Here's what they need to see.

On-screen: “The silent watchers”

- Week 1: discovery. Weeks 2–3: trust (process, doctor, results). Week 4: proof + next step.
- Your content mix must cover every stage every week.
- Show the journey map.

CTA: Save this journey map. Diagram · MOFU

3.5.3 Price isn't why patients leave your consultation without booking, Doctor. Regret is.

On-screen: “Price ❌ Regret ✅”

- High-ticket patients fear choosing the wrong doctor more than the price.
- Content that reduces regret: candidacy, realistic results, an aftercare plan.
- Turn 'what if I don't like it?' into a video series.

CTA: Comment REGRET for the series outline. Talking head · MOFU

PILLAR 4 OF 10 · 10% OF POSTS

Doctor Personal Branding & Authority (Without Becoming an Influencer)

Doctors want respect, not fame. You show them how to become the go-to doctor in their city.

Research basis: Keyword clusters: 'Doctors personal branding', 'Dermatologist/Hair transplant/Cosmetic surgeons personal branding'. Competitors: Amra Farooq ('building insane personal brands is stupid simple', 'DM brand'), 'Cult personal branding', 'Doctor Mike makes $2.68M/month'. Their gap: promises without process, timelines or proof; influencer framing.

4.1 Authority, not influence

From your File 2 persona: doctors value expertise and reputation and don't want to look like influencers. No competitor addresses this tension directly.

4.1.1 Doctors, you don't need to become an influencer to grow your clinic.

On-screen: “Doctor ≠ influencer”

- Influencers sell attention; doctors sell trust.
- No dancing, no trends you're uncomfortable with.
- What authority content looks like instead.

CTA: Follow if you want respect, not fame. Talking head · TOFU

4.1.2 The difference between a 'famous doctor' and a 'trusted doctor' on Instagram.

On-screen: “Famous vs. trusted”

- Famous: big numbers, broad topics, low bookings. Trusted: local, specific, consistent.
- Which one fills your calendar.
- 3 signals of a trusted-doctor profile.

CTA: Save this. Comparison · TOFU

4.1.3 How senior consultants can post without feeling it's 'beneath them'.

On-screen: “Senior doctors, this is for you”

- Reframe: content is your consultation at scale.
- Formats that suit seniority: case reasoning, opinion, teaching.
- Your peers will respect it, and patients will find you.

CTA: Send this to a senior colleague. Talking head · TOFU

4.2 Positioning: what you want to be known for

Built on: 'Doctor-specific content strategy (content advice is built for creators, not doctors)', Maha Chalawi 'average vs. expert'. Your gap: a positioning exercise for doctors.

4.2.1 Doctors, if patients can't finish this sentence about you, your brand isn't working.

On-screen: “'Dr. ___ is the doctor for ___'”

- Pick 1–2 treatments or problems you want to own in your city.
- Everything you post should reinforce it.
- Examples for a derm, a cosmetic dentist and an aesthetic doctor.

CTA: Comment POSITION and I'll help you fill the blank. Talking head · MOFU

4.2.2 Why 'I do everything' is the worst positioning for a dermatologist.

On-screen: “Everything = nothing”

- Generalist messaging gets lost; specific messaging gets remembered.
- You can still treat everything; just be known for something.
- How to choose your signature treatment.

CTA: Save this. Talking head · TOFU

4.2.3 Your 3 brand associations: the exercise I do with every doctor before writing any script.

On-screen: “3 associations”

- Association 1: your clinical focus. 2: your philosophy (e.g. natural results). 3: you as a person.
- Every video maps to one association.
- Show a filled example.

CTA: Comment BRAND for the worksheet. Worksheet walkthrough · MOFU

4.3 Your story as a doctor

Built on: Amra Farooq 'This is my story :)' (no turning point), founder-story reels. Your gap: a story structure doctors can use themselves.

4.3.1 Doctors, patients don't connect with your CV. They connect with your story.

On-screen: “CV ❌ Story ✅”

- Why you chose this specialty. A patient who changed how you practise.
- Your philosophy in one sentence.
- Credentials support the story, not the other way around.

CTA: Follow for the story series. Talking head · TOFU

4.3.2 The 'why I became a dermatologist' video every derm should post, and how to script it.

On-screen: “Your 'why' video”

- Structure: moment → tension → decision → what it means for patients.
- Keep it 45–60 seconds.
- A sample script on screen.

CTA: Comment WHY for the script template. Template · MOFU

4.3.3 3 personal stories doctors can share without compromising professionalism.

On-screen: “Personal but professional”

- Your training journey, a lesson from a hard case (anonymised), why you started your clinic.
- What to avoid: patient details, complaints about colleagues.
- Where the line sits.

CTA: Save this list. List · TOFU

4.4 Consistency and volume

Built on: Mrinal Tiwari 'Instagram is a game of volume' (no cadence given), your 70/20/10 rule. Your gap: a realistic cadence for busy doctors.

4.4.1 How many times a week should a doctor actually post? Here's a realistic answer.

On-screen: “How often should doctors post?”

- Minimum to build trust vs. ideal for growth.
- Why batching makes daily posting possible.
- Consistency beats perfection, but quality still matters.

CTA: Follow for more. Talking head · TOFU

4.4.2 Why doctors quit Instagram after 3 months, and how to stop it happening to you.

On-screen: “The 3-month quit”

- Results compound; month 1–3 feels slow.
- Measure the right things early: DMs and profile visits, not followers.
- Systemise so content doesn't depend on your mood.

CTA: Send this to a doctor about to give up. Talking head · TOFU

4.4.3 The 70/20/10 rule for doctors' content.

On-screen: “70 / 20 / 10”

- 70% proven formats from top creators in your specialty.
- 20% doubling down on your own winners. 10% experiments.
- How to find proven formats (the 5x outlier rule).

CTA: Comment 70 for my research template. Talking head · MOFU

4.5 Doctor personal brand vs. clinic brand

Your own differentiator: personal branding for doctors AND clinic branding through 4–5 specialists. Competitors treat these separately.

4.5.1 Should you grow your personal account or your clinic's account, Doctor?

On-screen: “Personal vs. clinic account”

- Personal: trust and portability. Clinic: scale and team.
- The best answer for most clinic owners: both, connected.
- How to link them without duplicate content.

CTA: Comment BOTH for the setup. Talking head · MOFU

4.5.2 What happens to your clinic's brand if it's built only around you?

On-screen: “Owner-dependent brand = risk”

- Founder-led works, until you want time off or a second branch.
- Feature your specialists too, so the brand scales.
- That's why I build clinic brands around the doctors.

CTA: Follow for the clinic-branding series. Talking head · MOFU

4.5.3 How top clinics make every doctor on their team a recognisable face.

On-screen: “Every doctor = a face”

- Each specialist gets a clear identity and topic area.
- The clinic account becomes a 'team of experts', not a logo.
- Patients book the doctor they feel they know.

CTA: Comment TEAM if your clinic has 3+ doctors. Talking head · BOFU

PILLAR 5 OF 10 · 10% OF POSTS

Clinic Branding: Build Your Clinic Around Its Specialists

Your clinic-branding offer. Stop random flyers and build the clinic's content around 4–5 specialist doctors and their personalities.

Research basis: Keyword clusters: 'Medspa/aesthetic clinic social media marketing', 'clinic growth strategy / healthcare marketing', 'Dental practice branding (your brand is what patients say when you're not in the room)'. Competitors: generic clinic-marketing reels and 'Top 10 marketing channels'. Their gap: nobody teaches specialist-led clinic branding. You own this.

5.1 Why clinic flyers don't work

From your offer description: 99% of clinics post generic flyers made by generic agencies, with no medical research. This is the problem your clinic branding solves.

5.1.1 Clinic owners: 99% of clinics post the same flyers. That's why patients can't tell you apart.

On-screen: “Same flyers. Same clinics.”

- Discount graphics, stock photos, 'book now'. Every clinic posts them.
- Patients don't choose clinics; they choose doctors.
- What to post instead: your doctors.

CTA: Follow if your clinic deserves better content.

Talking head · TOFU

5.1.2 Your marketing agency isn't medical. Here's what that costs your clinic.

On-screen: “Generic agency = generic content”

- No medical fact-checking, risky claims, generic captions.
- Patients notice; so do regulators.
- What to ask your agency tomorrow.

CTA: Comment ASK for the 5 questions.

Talking head · MOFU

5.1.3 I replaced a clinic's 30 flyers with 30 doctor videos. Here's the difference.

On-screen: “Flyers → doctors”

- Show the before grid and the after grid.
- Engagement and DMs go up when real doctors appear.
- Use a client (with permission) or a mock-up.

CTA: DM CLINIC if you want this for yours.

Before/after grid · BOFU

5.2 Doctor personalities inside a clinic

Your model: 4–5 specialist doctors per clinic, each with a personality and specialty. No competitor in your data covers this.

5.2.1 Every doctor in your clinic should have a 'content identity'. Here's how to find it.

On-screen: “Each doctor = 1 identity”

- Specialty focus + personality (the calm one, the straight-talker, the educator).
- Topics they own, so no two doctors overlap.
- Show a 4-doctor identity map.

CTA: Comment MAP for the template.

Diagram · MOFU

5.2.2 Your 4 doctors shouldn't post the same content. Here's how I split it.

On-screen: “4 doctors, 4 lanes”

- Derm: acne and pigmentation. Aesthetic: injectables. Dentist: smiles. Laser: body and hair.
- Each lane builds its own trust.
- Together, the clinic covers every patient.

CTA: Save this structure. Talking head · MOFU

5.2.3 The doctor in your clinic who hates the camera can still be a star. Here's how.

On-screen: “Camera-shy doctor? No problem.”

- Voiceover + B-roll, Q&A format, consultation-mode filming.
- Pair them with a confident colleague.
- Personality comes through in clarity, not performance.

CTA: Send this to the camera-shy doctor in your clinic. Talking head · TOFU

5.3 Clinic content system

Built on: 'Clinic operating systems', 'Monthly content planning'. Their gap: no system for multi-doctor content. You give one.

5.3.1 How I run content for a clinic with 5 doctors without chaos.

On-screen: “5 doctors. 1 system.”

- One filming day per doctor per month, a shared calendar, one approval board.
- Doctors approve scripts in 48 hours.
- The clinic posts daily without anyone thinking about it.

CTA: Comment SYSTEM for the workflow. BTS screen recording · MOFU

5.3.2 Clinic owners: your content calendar should follow your revenue goals, not holidays.

On-screen: “Calendar = revenue plan”

- Start with: which treatments need more bookings this quarter?
- Plan doctor content around those treatments.
- Holidays are extras, not the plan.

CTA: Save this. Talking head · MOFU

5.3.3 What one month of clinic content looks like when it's built around specialists.

On-screen: “1 month, 5 doctors”

- Show a calendar: each doctor's lane, the treatment focus, the funnel stage.
- Mix: education, trust, proof, team culture.
- This is what my clinic clients get.

CTA: DM CALENDAR to see one for your clinic. Calendar walkthrough · BOFU

5.4 Clinic profile and team presence

Built on: 'Patient proof displays', 'Your brand is what patients say when you're not in the room'. Their gap: no structure for team-led profiles.

5.4.1 Your clinic's Instagram should feel like meeting the team before the visit.

On-screen: “Meet the team before the visit”

- Team highlights per doctor, pinned introductions, consistent faces.
- Patients arrive already trusting the doctor.
- Show a team-led profile layout.

CTA: Follow for clinic-branding tips.

Profile walkthrough · MOFU

5.4.2 Clinic owners: should each doctor have their own account?

On-screen: “One account or many?”

- Pros and cons: control, portability, effort.
- A hybrid model: clinic account + doctor collabs.
- What happens if a doctor leaves.

CTA: Comment ACCOUNTS for the decision guide.

Talking head · MOFU

5.4.3 Your clinic's visual identity in 2 colours and 1 font. Here's why that's enough.

On-screen: “2 colours. 1 font.”

- Consistency builds recognition faster than variety.
- Subtitles, thumbnails, B-roll style.
- Show a messy clinic grid vs. a consistent one.

CTA: Save this.

Before/after grid · TOFU

5.5 Clinic culture and behind the scenes

Built on: Maha Chalawi 'Medi-spa growth and luxury team culture' (no behaviours defined), 'Patient experience quality'. Your gap: show culture as trust content.

5.5.1 Patients book clinics that feel safe. Here's how to show that on Instagram.

On-screen: “Show 'safe'”

- Sterilisation, consultation process, team meetings, case discussions.
- Behind the scenes = trust.
- 5 BTS video ideas for any clinic.

CTA: Comment BTS for the 5 ideas.

List · TOFU

5.5.2 Your team culture is marketing. Most clinics never show it.

On-screen: “Culture = marketing”

- Morning huddles, training days, celebrating patient milestones (with consent).
- Patients feel the culture before they visit.
- It also helps you hire better staff.

CTA: Follow for more clinic ideas.

Talking head · TOFU

5.5.3 What a luxury clinic in Dubai shows on Instagram that most US clinics don't.

On-screen: “Dubai luxury vs. US clinics”

- Experience: arrival, privacy, personalised plans.
- Language: consultation-first, discretion.
- What any clinic can borrow.

CTA: Follow for more Dubai × USA comparisons.

Comparison · MOFU

PILLAR 6 OF 10 · 5% OF POSTS

Compliant & Evidence-Based Marketing for Doctors (UAE + USA)

Your most defensible gap. Doctors fear getting it wrong; you're the doctor who knows the rules in Dubai and the US.

Research basis: Keyword cluster: 'Dermatologist/Hair transplant/Cosmetic surgeons personal branding' ('Medical advertising compliance'). Competitors: Amra Farooq ('comment Checklist', withheld), Akash Gupta and Shivam Bansal (India/NMC only). Their gap: no UAE (DHA/DoH/MOHAP) or US (FTC/HIPAA/state boards) content. ⚠ Verify every rule against the regulator's current guidance before filming.

6.1 UAE rules for clinic marketing

No competitor in your dataset covers DHA / DoH / MOHAP. An open lane.

6.1.1 Dubai clinic owners: your Instagram ads may need approval first. What to check with DHA.

On-screen: “Dubai: check this first”

- Health advertising in the UAE is regulated, and some content may need approval.
- What to check: who approves, what counts as an advert, what's restricted.
- Educational, not legal advice. Confirm with DHA.

CTA: Follow for UAE clinic rules made simple. Talking head + carousel · MOFU

6.1.2 Dubai vs. Abu Dhabi vs. the other emirates: who regulates your clinic's marketing?

On-screen: “DHA · DoH · MOHAP”

- Which authority covers which emirate.
- Multi-emirate clinics must check each one.
- Keep approvals on file.

CTA: Save this map. Carousel · MOFU

6.1.3 UAE doctors doing paid brand posts: what to check about advertiser permits.

On-screen: “Paid posts in the UAE?”

- The UAE regulates paid promotional content by creators.
- What to check before accepting a brand deal.
- Point to official sources.

CTA: Send this to a doctor who does brand deals. Talking head · MOFU

6.2 US rules for clinic marketing

Not covered by any competitor in your dataset (they are India/UK focused).

6.2.1 US doctors: that patient photo you reposted might be a HIPAA problem.

On-screen: “Repost = risk?”

- Identifiable patient images and stories in marketing generally need written authorization.
- What the authorization should cover.
- Safer alternatives.

CTA: Save this before your next repost. Talking head · MOFU

6.2.2 Cosmetic dentists: be careful calling yourself a 'specialist' in your marketing.

On-screen: “Careful with 'specialist'”

- Many state boards restrict 'specialist' claims to recognised specialties.
- Cosmetic dentistry is commonly not one, so check your state.
- Safer wording examples.

CTA: Send this to a cosmetic dentist. Talking head · MOFU

6.2.3 Clinic owners: what the FTC's rules on reviews mean for how you ask patients.

On-screen: “Reviews: the rules”

- Fake reviews and review suppression are prohibited.
- Be careful with incentives; ask everyone.
- Never reveal patient details when replying.

CTA: Save this checklist. Carousel · MOFU

6.3 Before/afters and testimonials

Built on: Dr. Nickc 'perfect patient testimonial' (no consent detail), 'Patient filming ethics'. Their gap: consent and honesty missing.

6.3.1 Doctors: your before/afters might be hurting trust. Same lighting, same angle, real consent.

On-screen: “Honest before/afters”

- Same lighting, angle, expression; no filters.
- Written consent, and say the number of sessions and the time between photos.
- A misleading pair vs. an honest pair.

CTA: Save this standard. Split screen · MOFU

6.3.2 How to film a patient testimonial that's compliant in the US and the UAE.

On-screen: “Testimonial consent checklist”

- Written consent before filming; explain where it'll be used.
- Let the patient review it; a right to withdraw.
- Interview questions that get honest, specific stories.

CTA: Comment TESTIMONIAL for the checklist. Checklist walkthrough · MOFU

6.3.3 'Results may vary' isn't enough, Doctor. Here's how to present outcomes properly.

On-screen: “Beyond 'results may vary'”

- Context: sessions, timeframe, candidacy.
- Show a range, not just the best case.
- Honest is both compliant and persuasive.

CTA: Follow for ethical marketing that still sells. Talking head · TOFU

6.4 Evidence-based content

Built on: dermatology product reels flagged in your research as lacking evidence level and safety context. Your gap: visible fact-checking by a doctor.

6.4.1 I fact-check every script I write for doctors. Here's the process.

On-screen: “Doctor-reviewed. Every script.”

- Claim → source → wording check → compliance check.
- Screen-record one example claim.
- Why generic agencies can't do this.

CTA: DM REVIEW if you want doctor-reviewed content.

BTS screen recording · BOFU

6.4.2 Doctors, these viral skincare claims are hurting your credibility when you repeat them.

On-screen: “Viral ≠ evidence”

- 3 popular claims doctors repeat without checking.
- How to say 'the evidence is limited' without sounding boring.
- Credibility is your brand. Protect it.

CTA: Save and send to a colleague.

Talking head · TOFU

6.4.3 Why AI-written medical scripts are risky for your clinic.

On-screen: “AI scripts: the risk”

- AI can invent facts and miss local rules.
- Use AI for structure; doctors for facts and voice.
- Always fact-check before posting.

CTA: Follow for safe AI use for doctors.

Talking head · TOFU

6.5 Claims and wording

Built on: compliance white spaces across your research ('substantiate claims', 'avoid implying universal results').

6.5.1 Doctors: delete 'painless', 'permanent' and 'guaranteed' from your content today.

On-screen: “3 words to delete”

- Absolute claims are risky and usually untrue.
- Honest alternatives that still sell.
- 5 caption fixes.

CTA: Save this list.

List · TOFU

6.5.2 I rewrote 5 risky clinic captions into compliant ones that still convert.

On-screen: “Risky → compliant → still sells”

- Split screen: original vs. rewrite.
- Why each rewrite is safer and more believable.
- Examples for injectables, laser and dental.

CTA: Comment CLAIMS for 20 more rewrites.

Split screen · MOFU

6.5.3 Educational or promotional? Why the difference matters for your clinic's posts.

On-screen: “Education vs. advertising”

- Promotional content often carries more rules.
- Educational content builds trust without a hard sell.
- How to mix both safely.

CTA: Follow for more.

Talking head · MOFU

PILLAR 7 OF 10 · 10% OF POSTS

The Patient Leak: Why Clinics Lose Inquiries Before They Book

The bridge between your content and AI offers: 'Content gets you the opportunity; your systems decide what happens to it.'

Research basis: Keyword clusters: 'patient inquiry conversion', 'Ai receptionist for medical clinics' (front desk), 'patient acquisition strategies' ('the most expensive sound is the phone ringing', 'offer two slots', 'where are you losing patients'). Competitors: Sam R Patel (front desk, no fixes or metrics), Dr. Nickc ('comment FOLLOWUP', withheld). Their gap: no live tests, no scripts, no AI layer.

7.1 Speed to lead

Built on: 'MedSpa owners, if your front desk takes forever to reply...'. Your gap: real (anonymised) tests with timestamps.

7.1.1 I messaged 10 Dubai aesthetic clinics at 9 pm. Only a few replied before morning.

On-screen: “9:00 PM → ? replies”

- A genuine inquiry, no bookings made, all names anonymised.
- Timestamps and reply quality on screen.
- The clinics that reply fast, ask a question and offer slots win.

CTA: Comment LEAK and I'll test your clinic.

Experiment series · TOFU

7.1.2 Clinic owners: every hour you wait to reply, that patient is talking to another clinic.

On-screen: “Slow reply = lost patient”

- High-intent patients message several clinics at once.
- The first helpful reply usually wins.
- Fix: instant acknowledgement + a question + human follow-up.

CTA: Send this to your front desk manager.

Talking head · MOFU

7.1.3 Who answers your clinic's DMs at 11 pm, Doctor?

On-screen: “Who answers at 11 PM?”

- Patients research at night.
- No reply until morning = cooled intent.
- Options: on-call staff vs. AI first response with human handover.

CTA: Comment NIGHT for the after-hours flow.

Talking head · MOFU

7.2 DMs and WhatsApp

Built on: 'Handling patient price questions', 'Price-estimate inquiries', 'Leads slipping through the cracks (WhatsApp)'. Their gap: no exact scripts.

7.2.1 How your team replies to 'How much is Botox?' decides whether you get the patient.

On-screen: “'How much?' — 4 replies”

- ❌ Just a price. ❌ 'Come in for a consult.' ❌ No reply.
- ✅ Range + what affects price + a question + 2 slots.
- Show the screenshots.

CTA: Save this and train your team with it.

Screen rec + script · MOFU

7.2.2 UAE clinic owners: WhatsApp is your real front desk. Most clinics run it badly.

On-screen: “WhatsApp = front desk”

- Greeting with the clinic name, quick acknowledgement, a qualifying question.
- Templates, quick replies, labels for follow-up.
- Opt-in and privacy basics.

CTA: Follow for UAE clinic tips. Demo · MOFU

7.2.3 Stop sending price lists to patients who DM you. Send these 3 messages instead.

On-screen: “Price list ❌ 3 messages ✅”

- Message 1: warm reply + one question.
- Message 2: tailored info + a short doctor video.
- Message 3: 2 appointment options.

CTA: Comment SEQUENCE for the copy. Script on screen · MOFU

7.3 Front desk and phone

Built on: Sam R Patel (greetings, costly mistakes, exceptional calls) and 'Stop asking what time works'. Their gap: no fixes; blames the receptionist.

7.3.1 Your receptionist says 'When would you like to come in?' and it's costing you bookings.

On-screen: “Two slots > open question”

- Open questions cause delay and drop-off.
- 'Tuesday 4 pm or Thursday 11 am?'
- Role-play both.

CTA: Send this to your receptionist. Role-play · MOFU

7.3.2 I reviewed a clinic's new-patient call. The booking was lost in 4 places.

On-screen: “4 lost moments”

- Greeting, discovery, price handling, closing.
- What should have been said.
- Use only calls you have permission to share, or recreate them.

CTA: Comment CALL for the scorecard. Call review · MOFU

7.3.3 Clinic owners: your receptionist isn't the problem. Your system is.

On-screen: “Not the person. The system.”

- Overloaded teams miss calls; that's normal.
- Scripts, CRM tasks, reminders, AI first response.
- Support your team; don't blame them.

CTA: Tag a clinic owner. Talking head · MOFU

7.4 After the consultation

Built on: 'If I had to stop patients ghosting after consultation...', Dr. Nickc 'post-consult patient loss'. Their gap: no sequence, timing or scripts.

7.4.1 Doctors: patients who don't book after your consultation aren't 'no'. They're 'not yet'.

On-screen: "'Not yet' ≠ 'no'"

- Reasons: cost planning, fear, a partner's opinion, timing.
- Each needs a different follow-up.
- Without one, 'not yet' becomes 'never'.

CTA: Follow for the follow-up series. Talking head · MOFU

7.4.2 The day 3, 7 and 21 follow-up that recovers 'I'll think about it' patients.

On-screen: "Day 3 · 7 · 21"

- Day 3: answer their main concern. Day 7: a helpful doctor video.
- Day 21: a gentle check-in + availability.
- The messages on screen.

CTA: Comment FOLLOWUP for the messages. Timeline graphic · BOFU

7.4.3 Clinic owners: there's revenue sitting in your 'pending treatment plans' list.

On-screen: "Your hidden revenue list"

- Every clinic has unfinished treatment plans.
- Pull the list, segment it, re-invite personally.
- Often the fastest win.

CTA: DM PLAN to have this run for your clinic. Talking head · BOFU

7.5 Leak audits

Built on: 'Where are you losing patients?' (unverifiable). Your gap: a visible, repeatable audit.

7.5.1 Doctors, here are the 7 places your clinic loses patients, from content to return visit.

On-screen: "7 leak points"

- Content → inquiry → response → booking → show-up → follow-up → return.
- One typical leak at each point.
- Score yourself out of 7.

CTA: Comment MAP for the self-audit. Diagram · MOFU

7.5.2 I audited a cosmetic dentist's patient journey. These were the 3 biggest leaks.

On-screen: "Audit: 3 leaks"

- Use a consenting client or an anonymised example.
- Evidence for each leak.
- The fix for each.

CTA: Comment AUDIT for yours. Teardown · BOFU

7.5.3 5 questions to find out if your clinic has a follow-up problem.

On-screen: "Follow-up problem?"

- Who follows up? When? How many times? Is it tracked? What happens after hours?
- 2+ 'I don't know' = a leak.
- What to fix first.

CTA: Save and answer honestly. Carousel · MOFU

PILLAR 8 OF 10 · 15% OF POSTS

AI Systems for Clinics: Booking, No-Shows & 24/7 Response

Your second authority pillar. AI always tied to a clinic outcome, with clinical guardrails from a doctor.

Research basis: Keyword clusters: 'Ai receptionist for medical clinics', 'healthcare chatbot / AI lead qualification', 'AI appointment booking', 'patient booking automation / no-show rate'. Competitors: Shivam Bansal (AI demo, no safety), Dr. Nickc ('build systems around AI', no demo), generic 'AI replaces receptionist' reels. Their gap: no medical privacy, escalation or proof.

8.1 AI receptionist and missed calls

Built on: 'A client calls at 8 pm; no one picks up', 'Missed calls are missed revenue' (not medical-specific). Your gap: a medical demo with guardrails.

8.1.1 Clinic owners, watch an AI receptionist answer a Dubai clinic's call at 11 pm in Arabic and English.

On-screen: “11 PM call → answered”

- A live demo (demo clinic): greeting, AI disclosure, question, booking.
- The language switch.
- Handover to a human for medical questions.

CTA: Comment AI to see it on your clinic's number.

Live demo · BOFU

8.1.2 If you set up one AI system in your clinic this month, make it this one.

On-screen: “Start here: missed-call text-back”

- A missed call triggers an instant text: 'Sorry we missed you, how can we help?'
- Cheap, fast, and recovers patients who'd otherwise call the next clinic.
- Show the flow.

CTA: Save this. Start here.

Demo · MOFU

8.1.3 As a doctor, here's what I never let an AI receptionist say to your patients.

On-screen: “AI red lines”

- Never diagnose, never advise, never promise results.
- Always disclose it's AI; escalate urgent or upset patients.
- Why a doctor should design these rules.

CTA: Follow for safe clinic AI.

Talking head · MOFU

8.2 Lead qualification and booking

Built on: 'Stop making your closer call 100 people to book 2', 'Fix your revenue by asking these questions'. Their gap: no rubric or medical use case.

8.2.1 An AI that asks the 4 questions your front desk forgets, then books the slot.

On-screen: “4 questions → booked”

- Treatment interest, timeline, previous treatments, preferred times.
- The answers go to your CRM, so you walk in prepared.
- Demo the chat.

CTA: Comment QUALIFY for the question set.

Demo · BOFU

8.2.2 Clinic owners worry AI sounds robotic. Here's a veneer inquiry handled naturally.

On-screen: “Human-sounding AI”

- Your clinic's voice, short messages, empathy.
- Robotic vs. natural, side by side.
- Always offer 'talk to the team'.

CTA: Follow for the next demo. Chat demo · MOFU

8.2.3 From Instagram DM to booked appointment in 40 seconds, with no staff involved.

On-screen: “DM → booked in 40s”

- Screen-record the flow on a demo account.
- Show the confirmation and reminders.
- This is what a 'system' means.

CTA: DM FLOW to get this built. Screen recording · BOFU

8.3 No-shows and cancellations

Built on: 'Stop sending basic appointment reminders', 'That confirmation message is vital', '5 steps to stop patients cancelling'. Their gap: no timing, copy or medical use.

8.3.1 Doctors, your reminder message might be why patients don't show up.

On-screen: “48h · 24h · Day-of”

- 48h: confirm + prep instructions. 24h: reminder + easy reschedule.
- Day-of: a friendly nudge + directions.
- The copy on screen.

CTA: Comment REMIND for the copy. Script on screen · MOFU

8.3.2 Clinic owners: turn every cancellation into a filled slot automatically.

On-screen: “Cancellation → filled”

- A cancellation triggers a message to the waitlist; first to confirm gets the slot.
- Show the flow.
- It protects your diary and revenue.

CTA: Follow for more clinic automations. Demo · BOFU

8.3.3 How to reduce no-shows without annoying your patients.

On-screen: “Fewer no-shows, happy patients”

- Deposits for high-value treatments.
- Easy 'reply YES' confirmation.
- Easy rescheduling, so a cancellation doesn't become a no-show.

CTA: Save this. Talking head · MOFU

8.4 Safe AI in healthcare

Built on: research white spaces repeatedly saying 'add privacy, consent, escalation, compliance'. Your doctor background makes this credible.

8.4.1 Clinic owners: AI should make your receptionist better, not replace them.

On-screen: “AI + receptionist”

- AI takes the repetitive work; your team handles people.
- Better patient experience, less burnout.
- How to introduce AI to your team.

CTA: Send this to your front desk team. Talking head · TOFU

8.4.2 US clinic owners: ask your AI vendor these HIPAA questions before you sign.

On-screen: “Ask your AI vendor”

- Will you sign a Business Associate Agreement?
- Where is data stored, who can access it, is it used for training?
- Deletion and breach response.

CTA: Save before you buy any AI tool. Carousel · MOFU

8.4.3 US clinics: automated texts and AI calls need patient consent. Here's what to know.

On-screen: “Consent first”

- US rules require consent for many automated texts and artificial-voice calls.
- Marketing messages need stronger consent than reminders.
- Educational, not legal advice.

CTA: Follow for compliant clinic AI. Talking head · MOFU

8.5 What to automate first

Built on: 'Using AI: red vs. green flag', 'Most AI in healthcare fails...'. Their gap: no examples or order of implementation.

8.5.1 Doctors: automate these 6 tasks in your clinic, and keep these 3 human.

On-screen: “Automate 6. Keep 3 human.”

- Automate: first response, FAQs, booking, reminders, follow-ups, review requests.
- Keep human: clinical advice, complaints, sensitive conversations.
- The two lists on screen.

CTA: Save this list. List · TOFU

8.5.2 An AI receptionist won't fix your clinic if your patient journey is broken.

On-screen: “Fix the journey first”

- Automating a bad process makes the mistakes faster.
- Map, fix, then automate.
- How I install systems.

CTA: Comment JOURNEY for the map. Talking head · MOFU

8.5.3 If I set up AI in your clinic, here's what I'd do in week 1, week 2 and week 4.

On-screen: “AI install roadmap”

- Week 1: missed-call text-back + instant DM reply.
- Week 2: booking + reminders. Week 4: follow-ups, reactivation, reviews.
- Quick wins first.

CTA: DM ROADMAP to get yours. Roadmap graphic · BOFU

PILLAR 9 OF 10 · 5% OF POSTS

Retention, Reactivation & Clinic Growth Numbers

The fastest ROI for established clinics, and the numbers that make you a strategic partner, not a vendor.

Research basis: Keyword clusters: 'patient communication automation & reactivation', 'patient retention strategies', 'patient database marketing', 'clinic google review / CRM', 'Clinic lead generation / ROI'. Competitors: Sam R Patel (retention rankings), Maha Chalawi (loyalty), Dr. Nickc (KPI teasers), Mrinal Tiwari (unverifiable case studies). Their gap: no segmentation, consent, cadence or math.

9.1 Database reactivation

Built on: 'Reactivate patients who haven't booked in 2 years', 'Best & worst ways to reactivate'. Their gap: consent, segmentation, cadence.

9.1.1 Doctors, you have patients who haven't visited in 12 months. Here's the first message I'd send them.

On-screen: “12 months away → message 1”

- Personal, from the doctor or clinic, not discount-first.
- Reference their last treatment and give a reason to return now.
- Easy reply + 2 time slots.

CTA: Comment REACTIVATE for the message. Script on screen · BOFU

9.1.2 Clinic owners: one 'we miss you' message to your whole list is a waste.

On-screen: “Segment first”

- Segment by treatment and time since the last visit.
- Check consent for marketing messages.
- Relevant = more replies, fewer unsubscribes.

CTA: Save this. Talking head · MOFU

9.1.3 The fastest revenue in your clinic is already in your patient database.

On-screen: “Your database = revenue”

- Past patients already trust you.
- Reactivation usually costs less than new acquisition.
- How to pull the list this week.

CTA: DM DATABASE to have this run for you. Talking head · BOFU

9.2 Rebooking and recall

Built on: 'A strong follow-up turns one visit into long-term patients'. Your gap: recall timing explained by a doctor.

9.2.1 Doctors, your patients forget to come back. Your system shouldn't.

On-screen: “Recall = system”

- Each treatment has a maintenance interval; follow your protocol.
- Too early feels salesy; too late and you lose them.
- Show a sample recall table.

CTA: Comment RECALL for the table. Talking head · MOFU

9.2.2 The day 1, 7 and 30 follow-up every aesthetic clinic should automate.

On-screen: “Day 1 · 7 · 30”

- Day 1: aftercare check (safety). Day 7: progress + questions.
- Day 30: result check + review ask + rebooking.
- Good care = good retention.

CTA: Save this sequence. Timeline · MOFU

9.2.3 Clinic owners: ask this question before every patient leaves.

On-screen: “Rebook before they leave”

- 'Your next session is ideally around [date]. Shall I hold a slot?'
- On-the-spot rebooking lifts return rates.
- Role-play it.

CTA: Send this to your front desk. Role-play · MOFU

9.3 Reviews and reputation

Built on: 'Google review request script', 'The first 50 reviews', 'QR-code reviews'. Their gap: compliance and automation.

9.3.1 Clinic owners: here's how to get more Google reviews without breaking the rules.

On-screen: “Reviews, the right way”

- Ask everyone, at the right moment, with a direct link.
- No fake reviews, no gating, careful with incentives.
- A script for your team.

CTA: Save this script. Script · MOFU

9.3.2 The automated review flow I set up for clinics.

On-screen: “Automated review flow”

- Trigger: visit completed + time for results to show.
- Thank you + a 1-tap review link; one reminder, then stop.
- Show the flow.

CTA: Comment REVIEWS for the flow. Demo · BOFU

9.3.3 Doctors: how to reply to a 1-star review without breaching confidentiality.

On-screen: “Replying to 1★”

- Never confirm they're a patient or mention treatment.
- Calm, brief, take it offline.
- A bad reply vs. a good reply.

CTA: Save this for the bad days. Talking head · MOFU

9.4 Clinic KPIs and ROI

Built on: '7 metrics every clinic owner should track', Dr. Nickc KPI teaser, 'New-patient acquisition math'. Their gap: KPIs not named, math hidden.

9.4.1 Clinic owners: the 6 numbers you should know every Monday morning.

On-screen: “6 Monday numbers”

- New inquiries, inquiry → booking %, show rate, new patients, rebooking rate, reviews.
- Define each in one line.
- Show a simple dashboard.

CTA: Comment KPI for the dashboard. Carousel + talking head · MOFU

9.4.2 Is your problem marketing or follow-up? One number tells you.

On-screen: “Marketing or follow-up?”

- Lots of inquiries + low booking rate = follow-up problem.
- Few inquiries + high booking rate = visibility problem.
- Fix the right thing first.

CTA: Save this diagnostic. Whiteboard · MOFU

9.4.3 Is a $2,000/month content + AI system worth it for your clinic? Let's do the math.

On-screen: “$2,000/month: worth it?”

- Average case value (assumption) → patients needed to break even.
- Add reactivated patients and recovered missed calls.
- Plug in your own numbers.

CTA: Comment ROI for the calculator. Whiteboard · BOFU

9.5 Growth decisions for clinic owners

Built on: 'You don't need a bigger team, you need better systems', Shivam Bansal (referral collapse), 'Top 10 marketing channels'. Their gap: no practical prioritisation.

9.5.1 Clinic owners: should you hire more staff or build systems first?

On-screen: “Hire or systemise?”

- Repetitive work → systemise. Clinical capacity → hire.
- Most clinics hire to fix a systems problem.
- A decision rule.

CTA: Save this rule. Talking head · MOFU

9.5.2 Doctors: if 100% of your patients come from referrals, your growth is left to chance.

On-screen: “100% referrals = risk”

- Referrals are great but unpredictable.
- Add a controllable channel: content + follow-up.
- Keep the referrals; add predictability.

CTA: Send this to a clinic owner. Talking head · TOFU

9.5.3 Launching a new treatment in your clinic? Here's my 30-day content + follow-up plan.

On-screen: “30-day treatment launch”

- Week 1: awareness. Week 2: who it's for + the doctor's view.
- Week 3: process + early results (with consent). Week 4: booking push + waitlist.
- AI handles inquiries throughout.

CTA: Comment LAUNCH for the plan. Roadmap · BOFU

PILLAR 10 OF 10 · 5% OF POSTS

Dr. Azka: A Doctor Building for Doctors (Story, Proof & Behind the Scenes)

Turns 'an agency' into 'the doctor I trust'. Your story, your journey and your client results.

Research basis: Keyword cluster: 'healthcare business founder story, doctor turned entrepreneur, clinic growth case study'. Competitors: Amra Farooq ('This is my story :)', no turning point), Shivam Bansal (inspirational, vague), Mrinal Tiwari and Joshua Navaro (case studies without verification). Their gap: no tension, milestones or verifiable proof. Your File 4 story has real tension.

10.1 Origin story

From your File 4: Healthcare → Doctor → Realisation → Experiments → Failures → AI → The gap → Helping doctors.

10.1.1 I'm a doctor at 24, and I chose to help doctors grow instead of practising, for now.

On-screen: "Doctor. 24. Different path."

- The realisation: being a great doctor isn't enough to fill a clinic.
- What you saw doctors struggle with.
- Why you decided to build this.

CTA: Follow the journey, Doctor.

Talking head · TOFU

10.1.2 Medical school taught me medicine. It never taught us how patients find doctors.

On-screen: "What med school didn't teach"

- The moment it clicked for you.
- Patients search online before they choose.
- You're building what you wish your colleagues had.

CTA: Tag a doctor friend.

Story + B-roll · TOFU

10.1.3 I didn't leave healthcare. I found another way to contribute to it.

On-screen: "Not leaving. Contributing."

- Better clinics = better patient access.
- Your work helps good doctors get found.
- Your mission line.

CTA: Follow if you believe this too.

Talking head · TOFU

10.2 Failures and lessons

From your File 4: e-commerce attempt, previous agency model. 'My failures are actually content.'

10.2.1 My first agency failed. Here's what I got wrong, so you don't hire an agency that makes the same mistakes.

On-screen: "My first agency failed"

- Too broad, wrong offer, no proof.
- What you learned about specialising.
- Why you now work only with clinics.

CTA: Follow for honest lessons.

Story · TOFU

10.2.2 What e-commerce taught me that most clinics still don't do.

On-screen: “E-commerce → clinics”

- Reply speed, follow-up, retargeting.
- Clinics rarely apply these.
- The bridge to your AI systems.

CTA: Save this lesson. Story · TOFU

10.2.3 Why I stopped working with every business and chose only clinics.

On-screen: “Only clinics. Here's why.”

- You understand doctors from the inside.
- Specialising = better results + higher trust.
- Your focus: derm, cosmetic dental, aesthetics.

CTA: Follow if you're a clinic owner. Talking head · MOFU

10.3 Building in public

Your 2026–27 targets ($10–12k → $50k → $100k/month) make a natural public series. Share numbers you're comfortable with.

10.3.1 Day 1: taking my clinic growth agency from $0 to $10k/month.

On-screen: “Day 1: $0 → $10k”

- The goal, the offer, the 60-day plan.
- Weekly updates.
- Real, not hype.

CTA: Follow for weekly updates. Series · TOFU

10.3.2 I messaged 100 dermatologists. Here's what they told me about their marketing.

On-screen: “100 dermatologists. Insights.”

- The most common frustrations.
- What they've tried that didn't work.
- What it means for clinic owners watching.

CTA: Comment 100 for the full findings. Story + data · MOFU

10.3.3 Month 1 update: clients, mistakes and what clinics are actually struggling with.

On-screen: “Month 1: the truth”

- Numbers you're comfortable sharing.
- One mistake, one win.
- The pattern you're seeing across clinics.

CTA: Follow for month 2. Series · MOFU

10.4 Behind the scenes of the work

From your File 4: 'Day in my life as a doctor building a business for doctors.' Show the work that justifies the price.

10.4.1 Behind the scenes: how I script 40 videos for one doctor.

On-screen: “40 scripts, behind the scenes”

- Research, outliers, fact-checking, hooks.
- Shows the work behind the result.
- A timelapse.

CTA: DM SCRIPTS if you want this for your clinic.

BTS · MOFU

10.4.2 A filming day at a client's clinic, from our side.

On-screen: “Filming day 🎬”

- Setup, the doctor's nerves, the shot list, the wins.
- How little time the doctor spends.
- Client permission required.

CTA: Follow for the results.

BTS vlog · MOFU

10.4.3 A day in my life as a doctor running a content + AI agency for clinics in Dubai.

On-screen: “Doctor. Founder. Dubai.”

- Client calls, scripting, AI builds.
- One lesson for clinic owners at the end.
- Purple-and-white aesthetic.

CTA: Follow for more.

Vlog · TOFU

10.5 Proof and client results

Competitors' case studies lack baseline, timeframe and attribution. Yours will include all three (from your pilot).

10.5.1 60 days with a [specialty] clinic: the baseline, what we changed, what happened.

On-screen: “Day 0 → Day 60”

- Baseline numbers, the changes, results with a timeframe.
- How results were attributed.
- Client permission required.

CTA: DM PILOT if you want to be next.

Case study · BOFU

10.5.2 The one video that brought my client their most valuable consultation.

On-screen: “1 video. 1 big patient.”

- The video, its hook and structure.
- How the patient found it.
- How to make more like it.

CTA: Comment WINNER for the template.

Breakdown · BOFU

10.5.3 What didn't work in our first clinic pilot, and what we changed.

On-screen: “What didn't work”

- Honest lessons build trust with doctors.
- One content lesson, one AI lesson, one process lesson.
- You learn fast, and that's the point.

CTA: Follow the build.

Honest recap · BOFU